

Simplifying SLMS around one resident, one journey, one care record.

A full review of every function and sidebar across all 19 roles — judged by what each screen actually does with data and workflow, not by its label. The goal: fewer, clearer paths to the work that matters, with every record, approval, audit trail, and clinical rule left intact.

Scope: 19 role portals · ~150 routed functions

Method: purpose + data + workflow, per screen

Prepared: 3 Sep 2026

● Draft for review — no changes made yet

01 Executive summary

The engine underneath is sound. The friction is on the surface: a few high-traffic roles carry very long sidebars where the same clinical job is split across many separately-named entries. Harmonizing those into a handful of hubs is a naming-and-grouping change, not a rebuild.

38

links on the Care Manager sidebar — the longest in the system. Nurse follows at 37.

7

overlap clusters where 2–6 entries do one clinical job (alerts, care docs, meds, assessments, plans, staffing, endorsement).

34

mostly-ungrouped entries on the Administrator sidebar, mixing clinical, platform & system tools.

0

functions, records, or routes deleted. Every change is relocate, rename, or group.

☒ Retain

☒ Combine

☒ Relocate into resident record

☒ Hide from routine users

☒ Remove from sidebar (route kept)

Every disposition below preserves the underlying screen and its data. “Remove from sidebar” means the link leaves the menu; the route stays reachable — the same pattern already used in the August 2026 sidebar curation.

02 Harmonizing similar functions

Seven clusters where the client's named themes — alerts & escalations, care documentation, medication, assessments, care plans, staffing — are currently spread across multiple sidebar entries. Each becomes a single hub with tabbed sub-views. Work queues stay one click away.

H-1 Alerts & escalations → one Action Queue

Four ways to "something needs attention" sit side by side on the nurse sidebar.

TODAY — 4 ENTRIES

- Clinical Alerts `alertcenter`
- Call Bells `callbells`
- Nurse Review & Escalations `escalations`
- Incident Review `incidents`

PROPOSED — 1 HUB, TABS

- **Action Queue** with tabs: Alerts · Call Bells · Escalations · Incidents
- One unread badge = total items awaiting the nurse
- Caregiver's "Escalate / Call Bell / Report Incident" collapse the same way

Why: all four are inbound work items triaged the same way. A single queue with a live count is faster to scan than four menu items with four badges. **Preserved:** incident records, escalation (SBAR) threads, and alert rules are unchanged — only the entry point merges.

■ Combine 4 → 1

■ Records intact

H-2

Care documentation & monitoring → one charting surface

Six monitoring boards slice the same 14 assessment domains into separate sidebar items.

TODAY — 6+ ENTRIES (NURSE)

- Daily Care Logs `carelogs`
- Daily Living / ADL `adlmonitoring`
- Weight Tracking `weightmonitoring`
- Wound Care `woundcare`
- Vital Sign Trends `vitalstrend`
- Domain Monitoring `domainmonitoring`

PROPOSED

- **Chart Care** — the daily log stays the shift charting entry
- ADL · Weight · Wound · Vitals · Domain become **tabs inside the resident record** (per-resident trends)
- Sidebar drops from 6 items to 1 charting entry

Why: ADL, weight, wound, vitals and domain scores are all per-resident trend views of the same assessment framework — they belong **in the resident's record**, reached from the directory, not as six parallel menu items. Shift-level charting (the daily log) stays on the sidebar because it's the frequent work queue.

Preserved: every board, its history, and the domain-monitoring discrepancy rules stay exactly as built.

■ Combine

■ Into resident record

■ Rules intact

H-3 Medication → one Medication hub

Administration, compliance, two stock lists, and orders are five-to-six separate entries per clinical role.

TODAY (NURSE / PHYSICIAN / CM)

- MAR `mar`
- Med Compliance `medcompliance`
- Med Inventory `medinventory`
- Mini Pharmacy `minipharmacy`
- Orders & Prescriptions `orders`
- Clinical Approvals `approvalworkflows`

PROPOSED — MEDICATION HUB

- Tabs: **Administer (MAR) · Compliance · Orders · Approvals**
- **Inventory + Mini Pharmacy merge** into one Stock tab (both are med stock)
- MAR — the daily work queue — remains the default landing tab

Why: these are stages of one medication lifecycle. Med Inventory and Mini Pharmacy are two stock ledgers that already cross-post charges. **Preserved:** MAR time-window rules, administration records, PIN-gated add-medication, and approval history are untouched.

■ Combine 6 → 1 hub

■ MAR queue stays top-level

H-4 Assessments → one Assessment & Level of Care hub

The same LOC engine is reached under three different names and routes.

TODAY — 3 LABELS FOR LOC

- Pre-Admission Assessment (v4.2) `prescreen`
- "Assessment & Level of Care" `rounds` / `casereview`
- Care Acuity & Level of Care `careacuity`

PROPOSED

- **Assessment & LOC** with **Pre-Admission (v4.2)** and **Reassessment / LOC Review** two clear entries:
- Retire the duplicate "rounds / casereview" label that points at the same board

Why: three route names for one level-of-care decision confuse which is authoritative. **Preserved — non-negotiable:** the approved **Assessment v4.2** scored engine, its 0–50 → Level 1–5 rules, and the validated Final-LOC authority all remain exactly intact; this is labeling only.

■ Merge labels

■ v4.2 engine intact

H-5 Care plans & routine → one Care Plan group

Plan, generator, and 24-hour routine are three adjacent entries.

TODAY

- Care Planning / Reviews `careplans`
- Care Plan Generator `careplangovernance`
- Routine Generator `routinenerator`

PROPOSED — CARE PLAN GROUP

- Tabs: **Plan & Review · Generate · 24-Hour Routine**
- PCG & Additional Clinical Services move under Services, not the plan menu

Why: generating a plan and its 24-hour routine is one continuous flow off the approved level. **Preserved — non-negotiable:** the Goals-of-Care framework, decision rules, and the 24-hour Routine Generator behave identically.

■ Group 3 → 1

■ GoC + 24h routine intact

H-6 Staffing → one Staffing group

Assignment, schedule, roster and profiles are four separate items that already feed each other.

TODAY

- Task / Caregiver Assignment `taskassignment`
- Caregiver Schedule `caregiverschedule`
- Staff Roster `staffroster`
- Staff Profiles & Records `staffprofiles`

PROPOSED — STAFFING GROUP

- Tabs: **Assignments** · **Schedule** · **Roster** · **Profiles**
- Assignments (the shift work queue) stays the default

Why: roster → schedule → assignment is one deployment pipeline. **Preserved:** the “only my residents during my shift” lock and roster import stay as built.

■ Group 4 → 1

H-7 Shift handover → one endorsement flow

Three end-of-shift entries where one handover flow would do.

TODAY (NURSE / CM)

- Shift Handover / Endorsements `shiftendorsements`
- Shift Documentation / Summary `shiftsummary`
- Endorsement Dashboard / Status `endorsementdashboard`

PROPOSED — SHIFT CLOSE

- Tabs: **Handover** · **What’s Charted** · **Status**
- Matches the caregiver “Shift Close” pattern already shipped

Why: the caregiver portal already groups these three under one “Shift Close” heading — this extends the same, proven pattern to nurse and care-manager.

■ Group 3 → 1

■ Carry-over & sign-off intact

03 One resident, one journey, one care record

The client's headline principle. Per-resident functions are scattered across the sidebar today; each should be a tab *inside* the resident record — which already exists as the **Resident Card (rcard)** and **One Care · One Journey** timeline. The sidebar keeps two doors in; everything resident-specific lives behind them.

SCATTERED PER-RESIDENT ENTRIES TODAY

- Resident Directory residents
- Clinical Records clinicalrecords
- Care Timeline / History carehistory
- One Care · One Journey residentjourney
- Vitals · ADL · Weight · Wound · Domain trends
- Vaccinations vaccinations
- Resident Documents documents
- Progress Report progressreport
- Care Plan · Assessment · MAR (per resident)

PROPOSED — SIDEBAR KEEPS TWO DOORS

- **Resident Directory** — the roster; a row opens the record
- **One Care · One Journey** — the full timeline
- Inside the record, tabs: **Profile · Journey · Clinical Records · Monitoring · Medications · Assessment & LOC · Care Plan · Documents · Vaccines**
- Every board becomes a resident-scoped tab, not a separate menu item

Why: a caregiver or nurse should reach everything about one person from that person's record — not hunt across eight sidebar entries and re-pick the resident each time. The record hub (rcard) and the journey aggregator are already built; this routes the scattered views into them. **Preserved:** all boards, CRUD, signatures, and Drive links are unchanged — they render inside the record instead of standalone.

■ Relocate ~8 entries into the record

■ All data & actions intact

04 Per-role before → after

Directional targets once the seven hubs and the resident record absorb their sub-entries. Counts are top-level sidebar items; grouped tabs are not counted. Frequent work queues (alerts, tasks, MAR, charting) stay at the top of each list.

ROLE	NOW		PROPOSED	CHIEF CHANGE
Care Manager Clinical oversight	38	→	~16	Med hub, staffing group, endorsement group, per-resident boards into record
Nurse Clinical care	37	→	~16	Action Queue, Medication hub, monitoring into record, Shift Close group
Administrator Full system access	34	→	~14	Split into Clinical / Operations / Administration / System groups; system tools hidden by default
Facility Admin Facility operations	24	→	~18	Already grouped; fold care-doc + inventory-alerts, tidy Settings
Physician Medical authority	24	→	~14	Medication hub, per-resident boards into record
Family Sponsor portal	20	→	~12	Resident-care views consolidate into the read-only journey/record
Caregiver Daily assistance	18	→	~14	Already curated (6 job-based groups); minor: monitoring into record
Resident My care	16	→	~10	Mirror family consolidation
Low-count operational roles — Billing (12), Fleet (15), Driver (11), Resident Coordinator (9), Platform / Org Admin (8–9), Security (4), Nutritionist (3), Housekeeping (2), Maintenance (2), Kitchen (1) — are already lean and change little, beyond dropping the boilerplate “Core SLMS Modules” block that appears on Fleet/Driver/Resident sidebars but doesn’t match those roles’ work.				

05 Technical & admin tools, out of the way

Configuration and platform screens should not compete for attention with clinical work queues. These stay fully available to the roles that own them — hidden from routine clinical sidebars.

TOOL	TODAY APPEARS ON	PROPOSED HOME	DISPOSITION
Geofencing geofencing	Administrator	System group (Admin only)	■ Hide
Camera Registry / Activity Log	Administrator, Facility, Security	Keep for Security; System group elsewhere	■ Hide from clinical
Landing Studio appearance	Administrator	System group (Admin only)	■ Hide
AI Assistant assistant	Administrator	System group / global launcher	■ Hide
SLMS Feature Matrix / Portal Matrix	Administrator	Administration group (Admin only)	■ Keep, grouped
Audit Log auditlog	Admin, CM, Physician	Administration group; CM keeps "Clinical Audit Trail"	■ Keep, grouped
"Core SLMS Modules" block	Fleet, Driver, Resident	Remove from these sidebars (not their job)	■ Remove from sidebar

Note: hiding is menu-level only. Audit trails keep recording, geofencing keeps enforcing, and every route stays reachable by permitted roles. Nothing is switched off.

06 What is preserved — the guardrails

Every non-negotiable the client named, held explicitly. This audit touches navigation, labels, and grouping only.



Assessment v4.2 engine

Scored 0–50 → Level 1–5, validated Final-LOC authority — unchanged.



Decision rules & domain triggers

14-domain scoring and discrepancy → LOC-review ladder behave identically.



Goals-of-Care framework

Care-plan generation off the approved level is untouched.



24-hour Routine Generator

Routine board and send-to-caregiver flow stay as built.



Care360 integration

Role-based dashboard mapping and lead intake preserved.



Records, approvals & audit trails

MAR, incidents, escalations, signatures, audit log — all retained.



No deletions

No function or historical data removed. Routes are kept even when a link leaves the menu.



Work queues stay fast

Alerts, tasks, MAR, and daily charting remain top-level, one click in.

07 Proposed rollout

Sequenced low-risk first, each phase independently shippable and reversible.
Nothing starts until you approve the direction.

1 Phase 1 — Grouping & labels

Config only

Collapse the seven clusters into hubs/groups and retire duplicate labels. Pure roleConfig naming and grouping; no board logic touched. Covers most of the visible simplification.

2 Phase 2 — Resident record consolidation

Routing

Route per-resident boards (monitoring, clinical records, vaccines, documents) as tabs inside the resident record. Sidebar keeps Directory + Journey.

3 Phase 3 — Admin/system tuck-away

Admin

Group Administrator into Clinical / Operations / Administration / System; hide system tools from clinical sidebars. Routes retained.

4 Phase 4 — Review & sign-off

Verify

Walk each role's new sidebar with you against real workflows; adjust before it's final. Existing role-sidebar tests extended to lock the agreed structure.

Awaiting your approval

This is a review draft — no navigation or code has been changed. The direction above keeps every record, approval, audit trail, and clinical rule intact, preserves Assessment v4.2, the decision rules, the Goals-of-Care framework, the 24-hour routine, and the Care360 integration, and deletes nothing.

To proceed, please confirm: (1) the seven harmonization hubs, (2) routing per-resident views into the resident record, and (3) the admin/system tuck-away — or mark any cluster to hold back. On approval we begin with Phase 1 (grouping & labels), which is reversible and touches no clinical logic.

Prepared for LifeCare Living Solutions · SLMS function & navigation review · 3 September 2026 · Sourced from the live role configuration across all 19 role portals. Counts reflect current sidebar entries; proposed counts are directional pending walkthrough.